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The History of the Asylum Revisited: Personal Reflections

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More than thirty years ago a colleague asked me a seemingly innocuous question about the history of psychiatry and the mentally ill. Intellectual curiosity then led me on a scholarly odyssey that began with a study of a single hospital and state, and concluded with a three-volume work that attempted to provide a comprehensive narrative of the ways in which American society managed the human, social, and economic problems associated with mental illnesses. At the time there was virtually no indication that the subject would become the focal point of a rich and controversial debate that served to act as a catalyst for research and a means of illuminating relationships between social structures and individual behavior. In this essay I should like to recount and to evaluate—employing both historiographical and autobiographical perspectives—the transformation of a marginal subject into one that has engaged historians, physicians, social and behavioral scientists, policy analysts, and public officials.

Before 1960 the study of mental illnesses and asylums was a relatively insignificant subject virtually lacking controversy. Many works dealing with the history of psychiatry were written by psychiatrists. Their celebratory tone indicated a desire to demonstrate the march of progress in their specialty. Gregory Zilboorg's well-known volume, which traced the evolution of psychiatric thought, was written with an inevitable Whig and historicist bias. To Zilboorg the history of psychiatry represented a secularizing movement from superstition to medical science. Nor was his approach unique. The volume of essays edited by J. K. Hall—the one-time president of the American Psychiatric Association—celebrated the centennial of the organization and embodied a similar theme, though in a somewhat more muted form. That these works had a Whiggish bias is not to suggest that they were bereft of value. On the contrary, they provided valuable information about a subject all too often ignored by mainstream academic historians. Nevertheless, the underlying commitment to study the history of psychiatry, institutions, and the mentally ill in terms of progress tended to make these works somewhat insensitive to contradictions, paradoxes, and the role of social, cultural, and intellectual factors.[1]

The most significant work on American psychiatry written within the Whig tradition was Albert Deutsch's classic *The Mentally Ill in America.* Paradoxically, Deutsch was not a physician. The son of Jewish immigrants, he grew up in harsh poverty and always identified with the poor and downtrodden. Although he never attended college, he was a voracious reader. While working on a project in the early 1930s that involved the collection of documents on the New York State Department of Welfare, he became fascinated with historical sources pertaining to the mentally ill. With verve and imagination, he persuaded Clifford W. Beers and the National Committee for Mental Hygiene to subsidize the research and writing of his book. When the completed manuscript was sent out for review to some prominent psychiatrists, profound differences emerged. The committee staff took the position that the manuscript was owned by the organization, which had final authority to make changes. In the end the differences were resolved, and the book appeared in 1937. Deutsch subsequently coauthored a history of public welfare in New York State, and during the 1940s and 1950s became the preeminent psychiatric and medical journalist of his generation. Indeed, his political liberalism and activist stance made him persona non grata to such AMA figures as Morris Fishbein, who regarded Deutsch as an exponent of Communist and Marxian ideology.[2]

The Mentally Ill in America was, in many ways, a tour de force. It reflected admirably the prevailing progressive ethos that dominated the writing of American history from the early twentieth century through World War II.[3] Written at a time when interest in or knowledge about the history of psychiatry and the mentally ill was nil, the work was based on primary sources that heretofore had not been used. Starting from scratch, Deutsch created an imposing historical framework that began with the travails of the mentally ill in colonial America when "punishment, repression and indifference" were characteristic. He then detailed the creation of mental hospitals in the early nineteenth century and their subsequent checkered development. Deutsch ranged widely over a variety of topics: concepts of mental illnesses, mental hygiene, and public policy. Committed to a historical tradition that emphasized progress, he nevertheless concluded that prevailing practices and policies still left much to be desired. Responsibility for existing shortcomings, he averred, rested with American society, which had failed to provide sufficient resources to meet the needs of the mentally ill. Even in his devastating exposés of public mental hospitals first published in the columns of the liberal newspaper *PM* in the mid-1940s and issued in book from in 1948, Deutsch did not despair about the future of institutional care and treatment. The "Ideal State Hospital," he insisted, was not a utopian dream, and he issued a clarion call for the public and their elected representatives to provide the needed resources.[4] In all of his work Deutsch combined historical analysis and a commitment to liberal reform, thus avoiding the extremes of either defending the status quo or rejecting psychiatry.

For more than a generation *The Mentally Ill in America* remained the definitive work on the subject even though few followed Deutsch's impressive beginning. Aside from some isolated doctoral dissertations, American historians manifested virtually no interest in the subject. They continued to emphasize political, military, and diplomatic topics. Even those who wrote about cultural and intellectual trends or reform movements rarely mentioned the mentally ill except in passing. Historians who helped to create the consensus school of American history after 1945—a school that minimized class divisions and emphasized the elements that united Americans—largely ignored

the mentally ill as well as other dependent and socially marginal groups that remained outside the mainstream.

During and after the 1960s, however, profound changes occurred in the writing of history. The general unrest arising out of war, racism, and poverty stimulated interest in these perennial problems. The growing popularity of the idea that all scholarship inevitably reflected ideological assumptions and commitments, moreover, led to an emphasis on the role of class and gender, and a proliferation of studies of "inarticulate" groups. Fascination with quantification and the influence of the social and behavioral sciences likewise reinforced interest in social structure—an approach that had already enjoyed a renaissance in Europe because of the influence of the French *Annales* school. Under the leadership of Lucien Febvre and Marc Bloch, members of this school attempted to obliterate traditional disciplinary boundaries in the hope of developing a more unified way of studying the experience of entire societies. The new social history, as it was often called, represented a peculiar amalgamation of scholarship, ideology, and activism. In the hands of some practitioners it became a surrogate for social if not revolutionary, change; in the hands of others it led to truly innovative scholarship.

Another source of change came from developments within the social and behavioral sciences. The rapprochement between psychiatry and the social and behavioral sciences, begun by Harry Stack Sullivan in the 1930s, continued after World War II. In the postwar decades the rise of the subdiscipline of medical sociology stimulated additional interest in mental illnesses and the role of psychiatry in America. That many historians were attracted to the subject was hardly surprising, given the fact that scholarly disciplines do not exist in an intellectual and cultural vacuum.

Out of this ferment came a distinctly different approach to the history of medicine. New social historians of medicine began to ask novel questions and explore hitherto neglected problems: the relationship between medicine and society; the interplay of popular and professional concepts of health and disease; the role of environmental and socioeconomic variables in changing morbidity and mortality patterns; the rise of new institutional forms and systems of treatment and care; and the ways in which medical interventions and technologies were actually employed. In so doing, they appreciably weakened the older commitment to Whiggish history and historicism. Less concerned with celebrating the progress of medicine, the new scholarship sought to understand the sources of change, the ways in which health care systems worked, and the uses to which such systems and technologies were put.[5]

Paradoxically, the greatest stimulus to research into the history of mental illnesses and psychiatry came not from historians but from social and psychiatric critics, including Michel Foucault, Thomas Szasz, Erving Goffman, and R. D. Laing. Save for their hostility to traditional forms of psychiatry, these individuals had little in common. Foucault flourished in a postwar revolutionary Parisian environment that had altered traditional forms of discourse and synthesized insights from Marx and Freud. Szasz was committed to a nineteenth-century classical libertarian ideology. Goffman was preoccupied with the overriding importance of personal autonomy. Laing's critique of psychiatry drew heavily on the social activism and political radicalism of the 1960s. Whatever their differences, these and other like-minded individuals adopted a hostile

stance toward psychiatry. Their work exerted a powerful influence on new social historians seeking to explain the origins of institutions dealing with deviancy.

In 1961 Foucault published his famous *Histoire de la folie* (translated into English a few years later in truncated form). This provocative and idiosyncratic book sought to describe the changing inner meaning of madness from the late middle ages to the birth of the asylum in the eighteenth century. Unlike liberal scholars who saw the asylum as a symbol of progress, Foucault had a quite different interpretation of this ubiquitous institution. The creation of asylums, he averred, represented an attempt to conquer madness by imposing a new system designed to enforce conformity. In Pinel's hands, he wrote, the asylum became ''an instrument of moral conformity and of social denunciation.'' The new understanding of insanity required that madpersons assume responsibility for their own condition. Nor was moral therapy a synonym for kind and humane treatment; it was rather an effort to force insane persons to develop an understanding of their own moral transgressions and then to alter their behavior by internalizing the values of their keepers. The physician thus became the ''essential figure of the asylum.'' His authority, however, did not derive from science, but rather from the moral and social order associated with bourgeois society and its values. Foucault's writings tended to demythologize psychiatry because of his insistence that the specialty's appeal was to be found not in its contribution to an understanding of human behavior, but in its relationships to the sources of power and domination.[6]

Ever-changing and often obscure, Foucault's writings became one of the pillars of dissenting and counterculture thought during and after the 1960s. His work seemed to strip away the melioristic and idealistic veneer of an alleged medical specialty and presumably benevolent institutions and lay bare their role in enforcing a universal conformity. By the late 1960s Foucault was one of the patron saints of a group of historians and social scientists determined to discredit the older idea that mental hospitals represented progress. Their scholarly studies often emphasized the social control functions of psychiatry and mental hospitals, the abuses inherent in institutionalization, and the demands generated by a capitalist social order that insisted on conformity to a unitary standard of citizenship and behavior.

Szasz, in contrast, was a practicing psychiatrist critical of most of his colleagues. Unlike Foucault, he was committed to a nineteenth-century liberalism that made individual liberty the paramount value. His work emphasized two distinct but interrelated themes: the ''myth of mental illness,'' and the role of psychiatry in suppressing nonconformity. In his eyes there were fundamental distinctions between the concepts of mental illness and conventional (i.e., organic) illnesses. Defined in conventional medical terminology, mental illness was a form of social labeling that had dramatic and drastic consequences for individuals. Indeed, psychiatry was little more than a pseudoscience. Its nosology lacked reliability and validity and embodied a set of value judgments that imposed a particular view of bourgeois reality (with all of its vested interests) upon a minority. Commitment laws, ostensibly intended to promote the welfare of patients, actually enhanced what Szasz called the ''Therapeutic State.'' Psychiatry, therefore, was merely an instrument of social control. As such, it constituted a threat to individual liberty in a free society precisely because of its rejection of such values as personal autonomy, volition, liberty, and responsibility. By the close of the decade Szasz's attack on the specialty had grown even more vituperative. ''To maintain that a social institution suffers from certain 'abuses,' '' he wrote in an ostensibly historical work, ''is to imply

that it has certain other desirable or good uses. This, in my opinion, has been the fatal weakness of the countless exposés—old and recent, literary and professional—of private and public mental hospitals. My thesis is quite different: Simply put, it is that there are, and can be, no abuses *of* Institutional Psychiatry, because Institutional Psychiatry *is*, itself, an abuse.''[7]

If Szasz attacked orthodox psychiatry because of his commitment to principles derived from nineteenth-century laissez-faire liberalism, figures such as R. D. Laing in England developed an equally powerful critique more compatible with the social activism and political radicalism of that decade. Laing, a practicing psychiatrist, was concerned with making intelligible the thoughts and feelings of schizophrenics, and subsequently with understanding their behavior within the context of disturbed interpersonal relationships in the family. Laing did not deny madness; he merely insisted that it was not incomprehensible. The basic difference between sanity and insanity was one of adaptation to social and cultural norms; those who refused to adapt were labeled insane. More importantly, Laing insisted that the concept of sanity was based on a statistical statement of normality. Those who defined insanity in such terms failed to question or to criticize dominant cultural values and therefore accepted an unsatisfactory status quo. ''There is no such 'condition' as 'schizophrenia,' '' he wrote in 1967, ''but the label is a social fact and the social fact a *political event*.''[8]

Goffman, on the other hand, was trained in sociology and wrote within the social interactionist tradition of George Herbert Mead. Given his belief in the overriding importance of personal autonomy, it was not surprising that he was most concerned with the impact of institutions such as mental hospitals upon the personality and behavior of patients. As a visiting scientist at the National Institute of Mental Health in Bethesda in the mid-1950s, he undertook field work at St. Elizabeth Hospital, a very large federal institution serving the District of Columbia. Out of his experiences came the publication of his famous *Asylums*. In this book he identified a ''total institution''—''a place of residence and work where a large number of like-situated individuals, cut off from the wider society for an appreciable period of time, together lead an enclosed, formally administered round of life.'' Goffman's concept of the total institution was by no means unique. Bruno Bettelheim's study of Nazi concentration camps (published in 1943) had already emphasized the ways in which extreme situations could shape and mold the collective behavior of inmates—a theme that resonated through some of the sociological and historical literature of the 1950s.

Goffman's own portrait of mental institutions was devastating; he described the ways in which a humiliating institutionalization stripped individuals of their self-identity and esteem and induced deviant responses. Expressions of hostility toward hospitals by patients were regarded as evidence that their commitment was proper and that they had not sufficiently recovered to be released. Mental hospitals, Goffman insisted, were staffed and administered in order to affirm the legitimacy of a medical service model. A paradox then followed. ''To get out of the hospital, or to ease their life within it, they [patients] must show acceptance of the place accorded them, and the place accorded them is to support the occupational role of those who appear to force this bargain.''[9]

Asylums was the product of a gifted and perceptive observer. Its literary and intellectual qualities were striking, and often overshadowed the methodological difficulties inherent in relying solely on personal observations. The book proved immensely popular among social activists, intellectuals, academics, and counterculture figures, many

of whom were persuaded that established institutions served only the rich and powerful. Curiously enough, Goffman's commitment to individual autonomy and a kind of eccentricity ensured that he would be far removed from many public issues. Nor did he offer any prescriptions for better and more effective ways of dealing with individuals designated as mental patients, and even conceded that the closing of all hospitals would "raise a clamor for new ones" by relatives and public authorities. His work, nevertheless, was absorbed into both popular and professional thought and became one of the most significant anti-institutionalist works of the 1960s.[10]

At about the same time that Goffman's critique appeared, others were developing themes that were implicitly or explicitly critical of psychiatry. During the 1950s sociologists by and large accepted the medical model of mental disorders; they were concerned with identifying etiological mechanisms, the extent of pathology in large populations, and the social response to pathology. A decade later epidemiological research was accompanied by an examination of the underlying concepts that shaped psychiatric thought and practice. In developing what became known as "labeling" (or societal reaction) theory, a number of sociologists called into question the very validity of the medical model of mental illnesses and, by indirection, the legitimacy of psychiatry as a whole.

Labeling theory did not emerge in an intellectual vacuum; sociological interest in the ways in which society identified deviant behavior had been longstanding. During the 1960s, however, labeling theory assumed new prominence, partly because it could be used as a form of social criticism. Thomas J. Scheff, a sociologist who helped to popularize labeling theory, argued that psychiatric diagnoses were merely convenient labels attached to individuals who had violated conventional behavioral norms. The breaking of norms, he emphasized, was common. Many violations, however, could not be placed within commonly recognized deviant categories (e.g., criminal, homosexual, vandal, etc.); mental illnesses therefore served as a residual category. Persons placed within this category in turn stimulated a response by agents representing dominant social groups. A label of mental illness, therefore, led to commitment to a mental hospital. In turn, the process of institutionalization altered self-conceptions and created a form of secondary deviance in which behavior reflected a new deviant self-image. The labeling or stigmatizing of individuals as mentally ill produced disturbed behavior. A diagnosis of mental illness, in other words, said less about the individual patient and more about the social system itself, the reaction of others to unconventional behavior, and the official agencies of control and treatment.[11]

The implications of labeling theory were obvious. Concepts of mental illnesses were not "neutral, value-free, [or] scientifically precise terms." They were, rather, as Scheff observed, "the leading edge of an ideology embedded in the historical and cultural present of the white middle class of Western societies." The real use of psychiatric labels such as schizophrenia was to reify and legitimate the existing social order. Psychiatrists were in reality fulfilling a social control rather than a medical purpose. Labeling theory was especially attractive to those who were critical of psychiatry and persuaded that fundamental social change was long overdue. Indeed, labeling theory proved appealing to a variety of activists, including civil rights advocates, feminists, and radicals of all persuasions.[12]

The powerful critiques developed by Foucault and others influenced historical scholarship. During the 1960s and 1970s a group of American historians (who constituted

only a small minority) were especially receptive to approaches that placed American, if not all of Western society, in a negative light. The emphasis on the persistence of racism, sexism, and poverty, when added to the controversy generated by the American involvement in Vietnam, quickly transformed the writing of history. The rise of the new social history, which was preoccupied with inarticulate groups, strengthened the conviction that its practitioners could play important roles in a forthcoming social transformation. By revealing the fundamental flaws in the American past, historians would prepare the foundation for the creation of a society in which inequality in all of its protean forms no longer existed.

The initial focus of new social historians was on race, gender, and class, which in turn stimulated interest in the history of welfare, dependency, and medicine. The emergence of the social history of medicine as a subspecialty in its own right was accompanied by a parallel interest in the history of mental illness and psychiatry. Yet the best-known work representing this new tradition—*The Discovery of the Asylum*—was not written by a social historian of medicine, but by David J. Rothman, a scholar whose initial interests lay in late nineteenth-century American politics. To be sure, Rothman was familiar with the field because his wife was a psychiatric social worker and their circle of friends included some psychiatrists. Nevertheless, his initial interest was with psychoanalysis; he hoped to pursue William L. Langer's call to apply psychoanalytic concepts to historical problems. Oscar Handlin, Rothman's mentor at Harvard, was less than enthusiastic. Discussions with Handlin in 1964 led Rothman to consider the possibility of studying the origins and rise of the asylum as an alternative to psychoanalytic history. During the early stages of his research, he quickly broadened his focus to include penal and welfare institutions that had some of the same characteristics as asylums.[13]

The Discovery of the Asylum, published in 1971, was a widely read and influential work focused on the origins of institutions of confinement in nineteenth-century America. Cognizant of his debt to Goffman in particular, Rothman developed a unique analysis that placed mental hospitals, prisons, and other institutions in a new light. The book embodied several related but distinct themes. In the early nineteenth century, according to Rothman, the stable society of colonial America was transformed. Economic and demographic changes in particular heightened concern with the social problems related to poverty, crime, indolence, and disease. Concerned with the need to restore social cohesion and obsessed with deviant and dependent behavior, Americans ultimately came to the conclusion that "to comprehend and control abnormal behavior promised to be the first step in establishing a new system for stabilizing the community, for binding citizens together." The solution, according to Rothman, involved the creation of the "asylum"—a unique institution designed to reform criminals, juvenile delinquents, poor and indigent groups, mentally ill persons, and all other deviants whose abnormal behavior threatened community stability and safety.[14]

Rothman, however, went beyond a study of origins. He presented a theory of the asylum that gave it a constant if not eternal character. Specifically, he insisted that the *character* of the asylum—its emphasis on authority, obedience, regularity, and discipline—ensured that it could never serve its intended purpose. Abuse, neglect, and inmate powerlessness were inherent in its very structure and organization. Indeed, Rothman seemed to imply that the development of the asylum followed an inexorable and predetermined pattern that mandated its custodial character for socially marginal groups.

That the book was written with a didactic purpose in mind was evident in its author's closing peroration, which both reflected and anticipated what sometimes is referred to as "deinstitutionalization."

> The history of the discovery of the asylum is not without a relevance that may be more liberating than stifling for us. . . . We tend to forget that they were the invention of one generation to serve very special needs, not the only possible reaction to social problems. . . . In this sense the story of the origins of the asylum is liberating. We need not remain trapped in inherited answers.[15]

The radicalism and anti-institutional ethos in certain intellectual circles during these years was also conducive to quasi-Marxian ideas. Andrew Scull—a sociologist by training who worked largely, if not exclusively, with historical data and materials—employed a quasi-Marxian analysis to delineate the functions and character of mental hospitals and psychiatry. Scottish-born, Scull grew up in a household committed to the Labor Party and left-wing politics. After graduating from Oxford, he came to Princeton to pursue graduate study; his interests lay in political sociology and particularly French politics. Since the Princeton sociology department had no specialist in that field, Scull enrolled in a seminar dealing with deviance and social control, and was introduced to Foucault and Rothman. His attention then shifted to mental illnesses, and he decided to undertake a dissertation that dealt with the origins of the asylum in England. Although focusing primarily on the English experience, he was attracted to a comparative approach. His articles and books offered a coherent, detailed, and logical history of the emergence of the asylum and the specialty of psychiatry.[16]

The origins of the asylum, according to Scull, lay in a series of "historically specific and closely interrelated changes." Specifically, an authentic shift in moral consciousness occurred. In the seventeenth century madness was equated with bestiality and a suspension of all rational faculties. Consequently, external discipline and compulsion were required to break the will of madpeople and to subjugate them. By the early nineteenth century, by contrast, madness had been domesticated. Lunatics were no longer perceived as beasts who lacked moral understanding; they were now perceived as having the same sensibilities as normal human beings. Hence they were receptive to appeals to reason and self-esteem. There was a shift, therefore, from external controls designed to enforce outward conformity toward an emphasis on the internalization of moral standards.

Such a change, Scull insisted, led unerringly to the creation of the ubiquitous asylum and the rise in the nineteenth century of what was known as moral treatment. This novel form of therapy was not an outgrowth of humanitarian sentiment; rather it was intended to transform lunatics and to remodel them "into something approximating the bourgeois ideal of the rational individual." Most important, the profound transformation in the perception and treatment of lunacy was a function of the rise of capitalism. "Just as those who formed the new industrial work force were to be taught the 'rational' self-interest essential if the market system were to work, the lunatics, too, were to be made over in the image of bourgeois rationality: defective human mechanisms were to be repaired so that they could once more compete in the marketplace."[17]

According to Scull, the rise of the asylum was also accompanied by the medicalizing of insanity and the emergence of the specialty of psychiatry. This development mirrored a larger social trend whereby elites rationalized and legitimated their control over devi-

ant and troublesome groups by assigning them to the authority of experts. The actual consequences of the medicalization of insanity and rise of the asylum, of course, were quite different. In short, asylums failed to live up to their promise and "remained a convenient place to get rid of inconvenient people."[18]

Scull's interpretation of recent developments in Anglo-American psychiatry followed along similar lines. Just as the asylum was the product of capitalism, so was deinstitutionalization. This new policy, which came into vogue during and after the 1950s, measured success by the decline in mental hospital inpatient populations and lengths of stay. Although its supporters idealized community care and treatment, reality was quite different. In practice, deinstitutionalization meant the transfer of aged mentally ill persons from hospitals to nursing homes, a policy that benefited entrepreneurs concerned with profits. Those who promoted community care, moreover, ignored "the reality of the increasingly segmented, isolated and atomized existence characteristic of late capitalist societies." In other words, just as capitalism had created the ubiquitous mental hospital in the nineteenth century, so, too, it abandoned its own creation after World War II.[19]

The heightened interest in psychiatry and the mentally ill paralleled the social and intellectual ferment of the 1960s and 1970s. During these years institutions and policies came under close scrutiny by scholars and activists, some of whom were critical of the ways in which public policies sometimes diminished individual and group rights. Demands for empowering those who were deemed disadvantaged also grew in significance. That historians would turn their attention to the roots of inequality and powerlessness was not unexpected. Equally understandable was the appearance of a body of historical literature critical of psychiatry and mental hospitals.

It was precisely at the beginning of the 1960s that I became involved with the history of psychiatry and the mentally ill. To have remained oblivious to the rich and stimulating scholarly controversies dealing with the origins and functions of asylums, the role of psychiatry, and the nature of the mentally ill population was inconceivable. Much of my own work, therefore, was inextricably bound up with the historiographical and policy debates of these years. Yet my original interest and involvement in this subject had somewhat different origins. This is not to suggest that I was able to transcend the influences that shaped other scholars. It is simply to assert that the forces that conditioned both my approach to and understanding of history were somewhat different.

Like many other American historians, I shared in the liberal ethos that dominated the discipline through World War II. I was born in 1931 at the beginning of the Great Depression. My parents were Polish Jews who migrated to the United States in 1922 and 1923 to escape a hostile environment. Married in 1929, they lacked any formal education. Nevertheless, they instilled in my sister and myself an almost naive faith in the redemptive authority of education quite apart from its role in enhancing career opportunities. Their Judaism, poverty, and liberal outlook made them staunch supporters of Franklin Delano Roosevelt and the New Deal. I can even recall when Roosevelt visited the Bronx on a rainy day during his reelection campaign in 1944, the thousands that lined the streets to pay him homage.

My commitment to the social democratic Left was strengthened during my years at the City College of New York. In the late 1940s City College remained a bastion of liberal if not radical thinking. A substantial part of its overwhelmingly Jewish student

body would have voted for Wallace in 1948 if they possessed the franchise (at that time those under the age of twenty-one years could not vote). The student body included many committed to Marxist and Communist ideals, which in their eyes provided an alternative to what appeared to be a rapacious capitalism already committed to a Cold War diplomacy. Although I was not unsympathetic to campus Marxists, their single-mindedness and hostility to alternative ideas proved unacceptable. My allegiances, therefore, remained with the liberal and social democratic Left.

My faith in a liberal political ideology and the promise of a better world, nevertheless, was always tempered by a recognition that human beings were neither completely rational nor moral. Too young to serve in World War II, I was increasingly aware of the horrors of nazism and the Holocaust. Not all members of my family had left Poland. With but a single exception, those who remained—including my grandfather—were murdered by the Nazis. The Holocaust left me with an abiding sense of tragedy and a recognition of human frailty. My approach to history, then, reflected a number of contradictory tendencies: a commitment to social democratic principles; a belief in the fallibility of human nature; a faith in the ability of individuals to make genuine choices that were in part independent of the forces operating upon them; and a hostility to any overarching historical explanations that bordered on determinism. Nor was I persuaded that all phenomena were linked or that it was possible to apply all-encompassing theories to human behavior and society. When introduced to the writings of Reinhold Niebuhr in the early 1950s, I immediately felt an affinity for the ways in which he combined conservative theology and radical politics while simultaneously warning of the dangers and pitfalls presented by the possession of power. Neibuhr employed irony and paradox without sanctifying or legitimating the claims of both the extreme Left and Right.

After graduating from City College in 1951, I briefly attended Columbia University. Receiving my MA in 1952, I left Columbia because I did not care for the factorylike atmosphere characteristic of its approach to graduate education. Through a circuitous route, I ended up at Northwestern University and received my Ph.D. in 1958. My dissertation dealt with late-nineteenth-century American trade unionism, a subject that had interested me since my undergraduate years at City College.

After two years of military service in the mid-1950s, I accepted my first teaching position at Clark University in Worcester, Massachusetts, in 1957. After my dissertation was accepted for publication, I began to search for a new research problem because I realized that my first book represented an institutional approach that was already dated.[20] Clark was then a very small school but one with a rich intellectual tradition. With a total faculty of no more than seventy, friendships transcended disciplinary affiliation. A colleague in psychology with extraordinarily broad interests suggested that I set a graduate student to work on the history of an old state mental hospital in Worcester. Quite ignorant about the subject, I nevertheless agreed to familiarize myself with the institution's past. Much to my surprise, I found that Worcester State Hospital had played an important role in the history of the care and treatment of the mentally ill. I also found that a large mass of manuscript material had survived, including every single patient case history since its formal opening in 1833 (which by the 1960s exceeded 70,000). I decided, therefore, to undertake myself a study of the history of the institution.

In retrospect, my decision to pursue research on the history of psychiatry and mental illnesses was somewhat presumptuous. My knowledge of both was nil, nor was I even

remotely familiar with a large social and behavioral science literature on the subject. Ignorance and fear of the unknown, however, did not prevail over what only can be described as a youthful hubris. I decided that with some work I could surmount my lack of knowledge, background, and training.

At the outset I pursued two related strategies. First, I began a systematic reading program in both psychiatry and the social and behavioral sciences. This proved an imposing experience, if only because of the unfamiliar terminology. Some of the figures who dealt with mental illnesses and psychiatry had a remarkable if misplaced affinity for dense and complex language and obscure terms. With a reasonable amount of hard work, however, I was able to familiarize myself with its underlying concepts (which often proved much more manageable than the language in which they were expressed). My second strategy, however, proved even more significant. In the early 1960s I attended on a fairly regular basis the training sessions offered to residents at the Worcester hospital in order to familiarize myself with clinical cases and institutional life. The experiences of being in contact with severely and chronically mentally ill patients led me to reject many of the social control and deviance theories and reflexive anti-psychiatric thinking that was just then emerging and becoming so prevalent.

Other elements also influenced my thinking at this time. I was not enamored with the ideology of the New Left and other activists that became so important for many scholars. Nor was I much influenced by some of the prevailing anti-institutional ideologies of these years. As far as the Vietnam War was concerned, I was a staunch opponent of American involvement as early as 1964 and signed one of the first anti-war advertisements in early 1965 that appeared in the *New York Times*. Nevertheless, I was quite disillusioned by what I felt was a partial abrogation of critical and careful thinking about social problems generally.

Consequently, I did not share the view that institutions—including mental hospitals—were inherently repressive (even though many in fact had such a character). My experience at Worcester State Hospital, of course, posed the risk of accepting conventional psychiatric claims at face value, but my reluctance to become involved in the ongoing policy debates dealing with the mentally ill helped me to retain at least a partial distance. The difficult and sometimes intractable nature of mental illnesses, moreover, led me to conclude that easy solutions were not readily available, and that most policy choices had at best ambiguous consequences. In this respect I was influenced both by Niebuhr's emphasis on human fallibility and Rene Dubos's attack on the illusion of medical utopias or disease-free societies.[21]

My book on Worcester State Hospital and public policy in Massachusetts was completed in 1965 and published the following year. At that time I was working in a partial historiographical vacuum. With the exception of Deutsch's classic work, relatively few scholars had worked on the history of psychiatry. Norman Dain's important study of early American psychiatric thought, completed as a doctoral dissertation in 1961, was not published until 1964.[22] The few institutional histories that had been published were written in filiopietistic terms. By the time I read Foucault's *Madness and Civilization* (which did not appear in translation until 1965), my book was largely finished. More important, I found Foucault's use of primary sources misleading and his writing extraordinarily dense and obscure. Although fascinated by many of his shrewd insights, I found the book to be neither logical nor coherent. Ultimately I came to the conclusion that

Foucault had to be read as a social critic rather than as a historian. Consequently, I was literally forced to create my own conceptual framework.

The writing of *The State and the Mentally Ill* presented formidable problems. I was determined not to write a purely local institutional history. Fortunately, the very process of research suggested some alternatives. While going through the hospital's own records, I became increasingly aware that its evolution and character were shaped in part by broader external considerations. This, in turn, led me to an examination of a category of sources all too often neglected, namely, Massachusetts state government records. The state, after all, was responsible for both the formulation and implementation of public policy. In the end I made a conscious decision to write what was in some respects a disjointed book that attempted to integrate the history of the hospital, the evolution of state policy, as well as an analysis of relevant external psychiatric, medical, social, and intellectual factors. My overriding concern, therefore, was with historical context and the ways in which seemingly disparate elements interacted. Moreover, I wanted to avoid the temptation of explaining events in purely logical terms. There is an important distinction, after all, between intentions and outcomes; to infer the former from a knowledge of the latter represents a form of ahistorical thinking.

A preoccupation with context, however, did not resolve complex interpretive issues; I found it virtually impossible to subsume institutional history within a one-dimensional framework. Institutional character and patient outcomes, for example, were by no means constant. Ethnicity and class, the process of professionalization, and ever-shifting state policies all interacted in subtle ways to give rise to unanticipated consequences. Well before Foucault, Rothman, and Scull published their works, I had already decided that the consequences of institutional care and treatment had varied sharply over time, and that it was all but impossible to assess the legacy of institutional care and treatment in simplistic or stark terms. The character of both individuals and institutions, I concluded, could not

> be easily portrayed or sharply delineated, for both contain elements of good and evil, white and black. This is particularly true for the history of psychiatry in general and mental hospitals in particular. Horace Mann and the other humanitarian reformers whose labors had resulted in the creation of Worcester State Hospital had envisaged a utopia of sorts, where an evil malady had once and for all been eradicated. Instead they found that for every problem solved and every obstacle overcome, two more had risen in their place. Like other institutions, the hospital had been influenced by prevailing social, economic, and intellectual forces; what had seemed worthy at one time had become less than ideal at another. . . . Perhaps the hospital had not accomplished all that its founders had hoped for, but surely it had done some things of value. By this standard the dreams of Mann and other reformers had not been in vain.[23]

The State and the Mentally Ill, though a narrow study, shaped virtually all of my subsequent thinking about the institutional history of American psychiatry. Even before its completion, I had decided to undertake a national study. In 1965 I had no indication that it would take more than twenty-five years and three volumes to complete the work. Nor did I envision the project as a response to the anti-psychiatry movement or to the subsequent writings of such scholars as Rothman and Scull. There is, I believe, a risk

in writing within a prevailing historiographical tradition; to do so permits the work of others to have an overly decisive role. Equally significant, I was concerned with process and change, and felt that many of the interpretations that were current were ahistorical in character. Moreover, the quest for overarching generalizations tended to homogenize the past and blur important changes over time.

I also felt that the process of research should play the primary role in shaping the questions ultimately asked by the historian. In emphasizing the significance of research in often obscure primary sources in shaping the evolution of my thinking, I do not mean to imply that the work of other scholars had no influence. Indeed, the historical and policy debates about the nature of institutions, the relative merits of institutional or community policies, and the role of psychiatry have always served as an indispensable intellectual stimulus. Yet a preoccupation with interpretive issues can lead to a neglect of sources that in turn suggest novel ways of dealing with familiar problems.

Let me offer several examples of how the process of research can assist in framing novel conclusions. Much of the scholarly and policy literature of the post–World War II era was based on the presumption that asylums functioned almost exclusively as custodial institutions; their therapeutic claims were dismissed as exaggerations if not overt falsifications. From this it was but a short step to emphasize the dehumanizing aspects of institutionalization on the personality and behavior of their inmates.

My research on the nature of the institutionalized population, however, led me to question formulations that made custodialism an inherent and seemingly eternal characteristic. I found, for example, that chronicity was not characteristic of hospital populations before 1890, when the typical length of stay (with some notable exceptions) was between three and nine months. The dramatic rise in the number of long-stay chronic patients began only in the 1890s. This finding directed my research in a new direction, namely, to identify the circumstances that transformed the nature of the institutionalized population.

Data from a variety of primary sources provided the basis for new kinds of formulations. I found that after 1890 aged senile patients (in addition to those whose abnormal behavioral characteristics had a somatic etiology) tended to be sent to mental hospitals instead of almshouses (which in the nineteenth century served as the old-age homes of that era). The transfer of aged patients grew out of a redefinition of senility in psychiatric terms by local government officials who took advantage of the passage of state care acts to shift fiscal responsibility for the aged from the local community to the state. As mental hospitals accepted ever-larger numbers of individuals over the age of sixty-five, their therapeutic functions declined correspondingly and internal environments changed sharply in less than positive ways. In an ideal world, of course, such a development would not have occurred. But in the real world, mental hospitals assumed a critical function—not necessarily willingly—of providing care and shelter for aged senile patients. Many of these individuals lacked access to basic care; some had no families and others had families who were unable or unwilling to assume responsibility. That other and perhaps better alternatives could have been found is besides the point; from a historical point of view it is important to recognize that reality does not always follow ideal patterns.[24]

Similarly, my research on the decades since World War II yielded some surprising findings. I found that the word generally used to describe these years–*deinstitutionalization*–was inappropriate if not misleading. Once again complexity, not simplicity,

was characteristic. The dissolution of the prevailing consensus around the wisdom of an institutionally oriented policy had a variety of sources. First, there was a shift in psychiatric thinking toward a psychodynamic and psychoanalytic model emphasizing the importance of prior life experiences as well as the role of socioenvironmental factors. Second, the experiences of World War II appeared to demonstrate the efficacy of community and outpatient treatment of disturbed persons. Third, the belief that early intervention in the community would be effective in preventing subsequent hospitalization became popular. Fourth, a faith developed that psychiatry could promote prevention by contributing toward the amelioration of social problems that allegedly fostered mental diseases. Fifth, the introduction of psychological and somatic therapies (including, but not limited to, psychotropic drugs) held out the promise of a more normal existence for patients outside of institutions. Finally, an enhanced social welfare role for the federal government not only began to diminish the authority of state governments, but also hastened the transition from an institution-based to a community-oriented policy. Throughout the postwar decades the rhetoric of community care and treatment and the alleged efficacy of various psychotherapies and, by implication, the obsolescence of mental hospitals, shaped public debates and agendas. That many of the rhetorical claims had little basis in fact was all but ignored by professionals, public officials, and especially the larger public.

My analysis of post–World War II developments, therefore, was framed in terms that did not break with the conceptual framework employed in *The State and the Mentally Ill*. The consequences of the innovations that transformed the mental health system, I wrote—like those of all human activities—were at best mixed. When the emphasis on treatment in the community was combined with an expansion of services to new groups, the result was a policy that often overlooked the need to provide supportive services for the seriously and chronically mentally ill. In this sense mental health activists all but ignored what their nineteenth-century predecessors had perhaps instinctively grasped, namely, that care and treatment, though conceptually separate, were not mutually exclusive. Thus the driving force of the postwar decades—antipathy toward traditional mental hospitals and a faith in community-oriented policies and institutions—gave rise to a bifurcated system with weak institutional linkages or mechanisms to ensure continuity and coordination of services. Severely and chronically mentally ill persons were released from public mental hospitals after brief lengths of stay into communities without adequate support mechanisms. The implications of the absence of longitudinal responsibility for meeting some of their basic needs in the community—including housing, medical care, welfare, and social support services—became painfully evident when the contraction of public welfare and housing programs exacerbated an already difficult situation.

In 1960 the history of psychiatry and mental illnesses as a subspecialty was virtually nonexistent. In ensuing decades a remarkable transformation took place. Within historical studies as well as the social and behavioral sciences, mental illnesses had become the focus of numerous and sustained inquiries. Interest in the subject, moreover, was not limited to scholarly discourse. The emphasis on civil rights fostered both a concern for the rights of the mentally ill and an interest in gender and class differences in the definition, identification, and treatment of mental disorders. The result was a lively, enlightening, if not fierce controversy.[25] Nor was the dialogue esoteric, if only because

particular historical interpretations were in turn employed to justify and to legitimate particular policies. To avoid involvement in these controversies was neither possible nor desirable; I was concerned that facile and attractive but inadequate historical explanations might become a justification for novel policies that promised more than it was possible to deliver. This is not to suggest that historical knowledge ought to have no role in policy debates. It is merely to insist upon the need to expose policymakers to the complexities and ambiguities of the past as well as to conflicting explanations. During the past two decades, for example, arguably questionable historical claims about mental hospitals were used to justify the alleged superiority of community-oriented policies.[26]

That I would find shortcomings in the work of other scholars was hardly surprising. Some lacked a firm empirical base; some rested on faulty logic; and others employed an overarching determinism that used the history of asylums and psychiatry to shed light on Western society. Above all, part of the historical literature was designed to undermine institutional care and psychiatric legitimacy or to provide a hostile critique of a capitalist social order. To be sure, no historian writes from a purely objective vantage point; basic assumptions and personal beliefs play a significant part in the manner in which reality is perceived and interpreted. To argue in this vein, however, is not to engage in reductionist thinking that rejects both the possibility of degrees of objectivity or insists that evidence matters but little.

Consider, for example, the controversy over the very definition of the "mental illnesses." Figures such as Szasz and Scheff emphasized the inability to identify physiological causes of mental disorders. From this they concluded that individuals designated as insane were in fact stigmatized and punished for their violation of conventional social norms. Foucault and Scull (who accepts the reality of mental disorders) went even further; they interpreted institutionalization as a way of disciplining idleness and nonproductivity. Within such a framework mental hospitals naturally assume an unyielding and eternal custodial and penal character; they merely symbolize the reactions of a bourgeois society committed to the ideals of rational production.

To equate psychiatric categories with stigmatization and deviance is understandable and perhaps to a certain extent logical. Yet to do so reduces psychiatric classifications to appendages of social, cultural, and intellectual factors. Admittedly, classification systems are neither inherently self-evident nor given. On the contrary, they emerge from the crucible of human experience; change and variability, not immutability, are characteristic. Indeed, the ways in which data are organized at various times reflect specific historical circumstances. Empirical data, after all, can be presented and analyzed in a variety of ways.[27]

To concede the arbitrary nature of classification, however, is not to maintain that psychiatrists were serving as agents of bourgeois capitalism by stigmatizing deviant and unproductive individuals. On the contrary, the evolution of psychiatric nosology involved the interplay of complex and subtle factors. Before the articulation of the specific germ theory of disease in the late nineteenth century, all physicians—psychiatrists or generalists—defined pathological states by describing them in terms of external signs and symptoms. This process was perhaps inevitable, if only because neither prevailing technology nor theory could establish a relationship between biological mechanisms and external and visible signs. Admittedly, a classification system based on external signs created formidable intellectual and scientific difficulties; the mid-

nineteenth century preoccupation with differentiating among various fevers was but one striking example of the problems faced by all physicians, general as well as psychiatric. Although disagreements on the diagnosis of signs and symptoms were common, few physicians questioned this approach, if only because other alternatives were largely absent. Nor was there any tendency to argue that individuals with a high fever were social deviants because their immediate condition (mental as well as physical) differed significantly from the population at large. Indeed, it is quite common at present to define pathological states even when patients appear to be in good health.

The psychiatric and medical definition of disease, moreover, was virtually identical; the inability of patients to function, combined with severe behavioral signs, was sufficient evidence to infer the presence of pathology. An explanation of disease often led to a rationale for treatment. In a certain sense psychiatrists were meeting a perhaps universal human need for explanations of unexpected or abnormal phenomena. Had they rejected this role, they could have fatally impaired their legitimacy.[28]

The theme of social control also resonates through part of the historical literature dealing with deviance and mental illnesses. The concept of social control has a long and somewhat checkered past since coming into vogue at the turn of the twentieth century. In 1901 E. A. Ross, an eminent sociologist, emphasized the tension between the behavior of individuals and the common interests and general welfare of society at large. In premodern societies, primary groups—family, church, local community—socialized and controlled the behavior of individuals. The progressive weakening of the authority of primary groups created a need for new forms of controls. Social control—the term Ross used to distinguish between premodern and modern societies—was a synonym for social welfare and implied that there had to be a balance between the desire for unregulated individual freedom and the needs of society for order and justice. As late as the 1930s Helen Everett identified social control with social planning and a rejection of laissez faire. By the mid-twentieth century, however, social control had acquired a new meaning; it now referred to the means employed by elites to perpetuate their hegemony and authority.[29]

Rothman's influential work owed much to social control theorists. In his view the invention of the asylum in the United States was a response to a perceived fear that hitherto stable social relationships were becoming unstable and thus posing a threat of social disorder. The asylum, paradoxically, sought to restore an older and presumably integrated community. If Rothman denied (somewhat unconvincingly) that his interpretation implied social control, Scull made this concept his central organizing theme. Thus the nineteenth-century emphasis on moral management "proved highly efficacious as a repressive instrument for controlling large numbers of people." In the twentieth century "the rise of the therapeutic state constituted a potentially massive expansion of psychiatry's role in the processes of social control." Indeed, psychiatry's "explanatory schema locates the source of pathology it identifies in intra-individual forces, and in principle can allow the redefinition of all protest and deviation from the dominant social order in individualistic and pathological terms."[30]

"Social control," of course, has a variety of meanings. That institutions and organizations control behavior is self-evident, if only in definitional terms. But is control the primary motive or is it a necessary by-product or concomitant? Surely it is evident that asylums restricted the freedom of patients. Yet asylums also served some of their patients in beneficial ways. Like most institutions, they included a variety of features.

What historians such as Rothman and Scull have done is to conflate intent and outcome. Implicit in their work is the belief that asylums were established to control deviant behavior and hence were the immoral products of a particular social system. To my way of thinking, social control is a given; it is inherent in the very definition of society. To use it in a pejorative way is to render the term all but meaningless.[31]

Equally important, asylums were hardly monolithic. The harsh, even terrifying, descriptions provided by Foucault, Rothman, and Scull tend to rest on a selective use of evidence; all data to the contrary are vigorously excluded. Rothman, for example, quotes early nineteenth-century asylum physicians to demonstrate the rigid, authoritarian, and repressive nature of moral treatment. To prove his point he italicizes key words in quotations such as "*perfect precision . . . neatness . . . orderly . . . systematic . . . regular.*" What this does is to alter in a subtle way the context of these remarks. What justification exists for such emphasis? Had physicians desired to single out these words, surely they would have done so (especially in light of the fact that italicization was very common in the midnineteenth century). Rothman could have just as easily italicized such words as *humane* and *kind*, but this would have contradicted his thesis. Scull uses evidence in much the same manner. And Foucault's use of data is so idiosyncratic as to make it virtually impossible to judge his work by normal historical standards. Moreover, these scholars make little or no use of patient correspondence, a source that provides a far more complex and ambiguous portrait of institutional life.

Nor do historical critics of asylums and psychiatry ever refer to outcome data. That the evaluation of institutionalization is extremely difficult is obvious. Nevertheless, admittedly imperfect data suggest that some individuals—whatever the reasons—were discharged from asylums in better condition than they entered. One late nineteenth-century study that followed about 1,000 patients discharged as recovered until they died found that about fifty-eight percent were never again hospitalized. Similarly, recent studies of nineteenth-century institutional outcomes by Nancy Tomes, Ellen Dwyer, and Constance McGovern—all based on a careful evaluation of surviving sources—demonstrated that institutions were hardly monolithic either in their internal character or outcomes. Twentieth-century data offer an equally variegated and ambiguous pattern. In an analysis of more than 15,000 patient cohorts admitted to Warren State Hospital between 1916 and 1950, Morton Kramer (a key figure in the collection and evaluation of psychiatric statistics) and his colleagues found large numbers of chronic and aged patients who remained institutionalized for long periods of time. But the study also found that the probability of release of first admissions of functional psychotics within twelve months increased from forty-two to sixty-two percent between 1919/1925 and 1946/1950. Such findings suggested that a high proportion of chronic and aged patients in public institutions led many to overlook the reality that substantial numbers of patients were admitted, treated, and discharged in less than a year.[32]

Other data also call into question the validity of historical interpretations that attribute to asylums and psychiatry invidious characteristics. If hospitals were agencies of social control, why were the majority of commitments initiated by families and not public authorities, including the police? Commitment, as a number of recent scholars have demonstrated, was overwhelmingly a family affair. In early twentieth-century San Francisco, for example, fifty-seven percent of commitments were inaugurated by relatives, twenty-one percent by physicians, and only eight percent by the police. The experiences of the Utica State Hospital and Pennsylvania Hospital for the Insane in the

nineteenth century were much the same. The familial decision to commit, Tomes noted in her study, generally came ''after a prolonged crisis of escalating tension and desperation culminated in a crisis.'' Only when the very integrity of the family was threatened did its members consider hospitalization, a decision that was made with considerable ambivalence and reluctance.[33]

Equally significant, historical critics rarely refer to the nature of the institutionalized population. Between 1890 and 1950, a large proportion of patients in mental hospitals had somatic diagnoses, including senility, cerebral arteriosclerosis, general paresis, cerebral syphilis, Huntington's disease, pellagra, or various brain diseases. Many of these patients required total care. Lacking other alternatives, it was understandable (even if undesirable) that mental hospitals assumed a caring or custodial function. Moreover, institutional character was not merely shaped by asylum physicians or elites; the natures of patients were an important determinant of internal environments. The hegemonic arguments of Foucault, Rothman, and Scull often neglect or overlook such evidence. Thus they write about *the* asylum as though differences were of no consequence whatsoever. If historians of American higher education were to write a history of *the* college and university, they would immediately come under criticism for their failure to distinguish between very different types of institutions. Why then should we accept a one-dimensional view of mental hospitals that ignores significant temporal and regional variations. Indeed, the very concept of a monolithic eternal asylum is akin to an ''ahistorical history''!

Admittedly, the coherence and symmetry of these reductionist interpretations are alluring. There are no inconsistencies; individual and collective behavior are explainable in rational terms; human volition, chance, and accident are largely absent; and the merging of historical data with social and behavioral science insights creates an all-encompassing structure capable of comprehending all of the details that constitute an approximation of social reality. The entire universe is linked within a unitary system that can be both understood and explained. Within this system all phenomena are linked in causal ways. The asylum and psychiatry are either shaped by an impersonal economic system and a commitment to productivity and the internalization of controls or by inexorable laws that govern all institutions.

Some years ago J. H. Plumb drew a sharp distinction between the past and history. History, he insisted, is not the past, even though they share common elements. History represents an effort ''to see things as they actually were, and from this study to formulate processes of social change which are acceptable on historical grounds and none other.'' The past, on the other hand, has always been ''a created ideology with a purpose, designed to control individuals, or motivate societies, or inspire classes.''[34] Plumb's distinction is particularly applicable to part of the scholarship pertaining to the history of the asylum and psychiatry, which is designed to rationalize the need for presumably fundamental social and economic change. The result is a scholarship that is partly distorted because of its didactic qualities. Fortunately, younger scholars writing about mental illnesses and psychiatry have moved away from the rigid ideological traditions so characteristic of the 1960s and 1970s.

In concluding, I would be less than honest if I did not concede that my own experiences and assumptions played a role in the ways in which I have written about asylums and psychiatry. I remain skeptical of the modern faith that human beings can mold and

control their world in predetermined and predictable ways. This is not to suggest that we are totally powerless to control our destiny. It is only to insist upon both our fallibility and our inability to predict all of the consequences of our actions. Nor do I believe that human behavior can be reduced to a set of deterministic or quasi-deterministic laws or generalizations, or that solutions are readily available for all problems. Tragedy is a recurring theme in human history and defines the parameters of our existence.

Historians must also be cognizant of the inherent dangers of arguing that the answers to contemporary problems resides in historical analysis. For if this were true, then surely our society should be governed not by Plato's proverbial philosophers or Lester Frank Ward's sociologists but by historians. A *historiocracy* (to coin a new word) would in all likelihood be no wiser than a sociocracy or aristocracy! In so arguing I do not mean to suggest that the study of history is of no value or importance. That historical knowledge shapes and is shaped by contemporary attitudes and behavior is to some degree obvious. All human beings employ history in one form or another to mold, validate, and justify their prescriptions for the present and future. The issue, then, is not whether historical knowledge will be used, but what kind of history.

Above all, those of us who write about the past must avoid, insofar as is possible, the seductive temptation to judge past events without at least a small measure of compassion and understanding as well as a frank recognition that we are not omniscient. If we fail to do so, our successors will be justified in judging our own generation in equally harsh terms.

Notes

1. Gregory Zilboorg, *A History of Medical Psychology* (New York: Norton, 1941) and *The Medical Man and Witch during the Renaissance* (Baltimore: Johns Hopkins Press, 1935); J. K. Hall (ed.), *One Hundred Years of American Psychiatry* (New York: Columbia University Press, 1944). Many standard works fell within this tradition, including *The Institutional Care of the Insane in the United States and Canada*, 4 vols. edited by Henry M. Hurd (Baltimore, Johns Hopkins Press, 1916–1917), William L. Russell's *The New York Hospital: A History of the Psychiatric Service 1771–1936* (New York: Columbia University Press, 1945), and Earl D. Bond's *Dr. Kirkbride and His Mental Hospital* (Philadelphia: J. B. Lippincott, 1947).

2. Jeanne L. Brand, "Albert Deutsch: The Historian as Social Reformer," *Journal of the History of Medicine and Allied Sciences*, xviii (1963), 149–157; Deutsch, *The Mentally Ill in America: A History of Their Care and Treatment from Colonial Times* (Garden City, New York: Doubleday Doran, 1937); David M. Schneider and Albert Deutsch, *The History of Public Welfare in New York State 1867–1940* (Chicago: University of Chicago Press, 1941); George Mora, "Three American Historians of Psychiatry: Albert Deutsch, Gregory Zilboorg, George Rosen" in *Essays in the History of Psychiatry*, edited by Edwin R. Wallace and Lucius C. Pressley (Columbia, South Carolina: William S. Hall Psychiatric Institute, 1980), 1–21. The American Foundation for Mental Health Papers at the Archives of Psychiatry, New York Hospital-Cornell Medical Center, New York, NY, contain correspondence between Beers, Edmund Bullis, and Deutsch over the issue of organizational censorship.

3. For an analysis of what has been termed the Progressive School of American Historiography, see John Higham, *History* (Englewood Cliffs, New Jersey: Prentice-Hall, 1965).

4. Deutsch, *Mentally Ill in America* and *The Shame of the States* (New York: Harcourt, Brace, 1948).

5. Cf. Gerald N. Grob, "The Social History of Medicine and Disease in America: Problems and Possibilities," *Journal of Social History*, x (1977), 391–409.

6. Foucault's numerous books include *Madness and Civilization: A History of Insanity in the Age of Reason* (New York: Pantheon, 1965), quote from p. 259, originally issued as *Folie et deraison, Histoire de la folie à l'âge classique* (Paris: Union generale d'editions, 1961); *The Order of Things: An Archaeology of the Human Sciences* (New York: Pantheon, 1970); *The Archaeology of Knowledge and the Discourse on Language* (New York: Pantheon, 1972); *The Birth of the Clinic: An Archaeology of Medical Perception* (New York: Pantheon, 1973); *Discipline and Punish: The Birth of the Prison* (New York: Pantheon, 1977).

7. Space precludes a complete listing of Szasz's numerous articles and books. For some representative writings in the late 1950s see "Commitment of the Mentally Ill: 'Treatment' or Social Restraint?" *Journal of Nervous and Mental Disease*, cxxv (1957), 293–307, and "The Classification of 'Mental Illness,' " *Psychiatric Quarterly*, xxxiii (1959), 77–101. His most controversial books published during the 1960s include *The Myth of Mental Illness: Foundations of a Theory of Personal Conduct* (New York: Hoeber-Harper, 1961); *Law, Liberty, and Psychiatry: An Inquiry Into the Social Uses of Mental Health Practices* (New York: Macmillan, 1963); *Psychiatric Justice* (New York: Macmillan, 1965); and *The Manufacture of Madness: A Comparative Study of the Inquisition and the Mental Health Movement* (New York: Harper & Row, 1970), quotes from pp. xix–xxv.

8. R. D. Laing, *The Politics of Experience and the Bird of Paradise* (London: Penguin Books 1967), 100; *The Divided Self: An Existential Study in Sanity and Madness* (London: Penguin, 1960), and (with A. Esterson) *Sanity, Madness, and the Family* (London: Tavistock, 1964).

9. Erving Goffman, *Asylums: Essays on the Social Situation of Mental Patients and other Inmates* (Garden City, New York: Doubleday, 1961), xiii, 385–386. See also Bruno Bettelheim, "Individual and Mass Behavior in Extreme Situations," *Journal of Abnormal and Social Psychology*, xxxviii (1943), 417–452, Gresham M. Sykes, *The Society of Captives: A Study of a Maximum Security Prison* (Princeton: Princeton University Press, 1958), and Stanley M. Elkins, *Slavery: A Problem in American Institutional and Intellectual Life* (Chicago: University of Chicago Press, 1959).

10. Goffman, *Asylums*, 384. The social and behavioral science and counterculture literature of the 1960s and early 1970s are replete with references to Goffman's work. For subsequent critical evaluations of Goffman's methodology and interpretations see Roger Peele, P. V. Luisada, M. Jo Lucas, D. Russell, and D. Taylor, "*Asylums* Revisited," *American Journal of Psychiatry*, cxxxiv (1977), 1077–1081; Peter Sedgwick, *Psycho-Politics* (New York: Harper & Row, 1982); and David Mechanic, "Medical Sociology: Some Tensions Among Theory, Method, and Substance," *Journal of Health and Social Behavior*, xxx (1989), 147–150.

11. Thomas J. Scheff's *Being Mentally Ill: A Sociological Theory* (Chicago: Aldine Publishing Co., 1966) was the classic statement of labeling theory. The concept, however, set off a heated and controversial debate among sociologists. See, for example, Walter R. Gove, "Societal Reaction as an Explanation of Mental Illness: An Evaluation," *American Sociological Review*, xxxv (1970), 873–884 (and the comments in ibid., xxxvii [1972], 487–490); Gove and Patrick Howell, "Individual Resources and Mental Hospitalization: A Comparison and Evaluation of the Societal Reaction and Psychiatric Perspectives," ibid., xxxix (1974), 86–100; and Scheff, "The Labelling Theory of Mental Illness," ibid., 444–452.

12. Scheff, "Schizophrenia as Ideology," *Schizophrenia Bulletin*, no. 2 (Fall 1970), 15–19.

13. David J. Rothman, *Politics and Power: The United States Senate, 1869–1901* (Cambridge: Harvard University Press, 1966). Biographical details from a conversation with Rothman on 2 April, 1992. William L. Langer's call for a psychoanalytic history was the theme of his presidential address at the American Historical Association in December 1957. It was published as "The Next Assignment," *American Historical Review*, lxiii (1958), 283–304.

14. David J. Rothman, *The Discovery of the Asylum: Social Order and Disorder in the New*

Republic (Boston: Little, Brown, 1971), 58–59. His sequel volume, *Conscience and Convenience: The Asylum and Its Alternatives in Progressive America* (Boston: Little, Brown, 1980), drew far less attention.

15. Rothman, *Discovery of the Asylum*, 295.

16. Scull's books include *Museums of Madness: The Social Organization of Insanity in Nineteenth Century England* (New York: St. Martin's Press, 1979) and *Decarceration: Community Treatment and the Deviant—A Radical View* (Englewood Cliffs, New Jersey: Prentice-Hall, 1977). Many of his articles are collected in his *Social Order/Mental Disorder: Anglo-American Psychiatry in Historical Perspective* (Berkeley: University of California Press, 1989). Biographical details were provided by Scull in a conversation on 1 April 1992.

17. Scull, *Social Order/Mental Disorder*, 43, 89, 94.

18. Ibid., 231, 305, 307.

19. Ibid., 314, 322–323, 327. For an extended treatment see also Scull's *Decarceration*.

Although concerned largely with indigency and dependency, Michael Katz also developed an interpretation of the asylum in the United States somewhat similar to that of Scull. Katz's most incisive interpretation appeared in his article "Origins of the Institutional State," *Marxist Perspectives*, i (1978), 6–22.

20. My dissertation appeared as *Workers and Utopia: A Study of Ideological Conflict in the American Labor Movement, 1865–1900* (Evanston: Northwestern University Press, 1961).

21. I was especially taken with Reinhold Niebuhr's *The Children of Light and the Children of Darkness* (New York: Scribner, 1944), although I had also read *Moral Man and Immoral Society* (New York: Scribner, 1932), *The Nature and Destiny of Man*, 2 vols. (New York: Scribner, 1942), and *The Irony of American History* (New York: Scribner, 1952). René Dubos's *Mirage of Health: Utopias, Progress, and Biological Change* (New York: Harper, 1959) was also crucial to my thinking; his subsequent book *Man Adapting* (New Haven: Yale University Press, 1965) filled in many details.

22. Norman Dain, *Concepts of Insanity in the United States 1789–1865* (New Brunswick: Rutgers University Press, 1964). Examples of older institutional histories included Hurd, *Institutional Care of the Insane* and Russell's *New York Hospital*. The essays in Hall, *One Hundred Years of American Psychiatry*, were of some value.

23. Grob, *The State and the Mentally Ill: A History of Worcester State Hospital in Massachusetts, 1830–1920* (Chapel Hill: University of North Carolina Press, 1966), 363.

24. Grob, *Mental Institutions in America: Social Policy to 1875* (New York: Free Press, 1973), *Mental Illness and American Society 1875–1940* (Princeton: Princeton University Press, 1983), and *From Asylum to Community: Mental Health Policy in Modern America* (Princeton: Princeton University Press, 1991). The delay in completing the second volume was due to the fact that I spent several years in completing a biography (*Edward Jarvis and the Medical World of Nineteenth-Century America* [Knoxville: University of Tennessee Press, 1978]).

25. See, for example, Phyllis Chesler, *Women and Madness* (New York: Doubleday, 1972), and Elaine Showalter, *The Female Malady: Women, Madness, and English Culture, 1830–1980* (New York: Pantheon, 1985). For an insightful review see Nancy Tomes, "Historical Perspectives on Women and Mental Illness" in *Women, Health, and Medicine in America: A Historical Handbook*, edited by Rima D. Apple (New York: Garland Publishing Co., 1990), 143–171.

26. My contributions to the debate were the following articles: "Welfare and Poverty in American History," *Reviews in American History*, i (1973), 43–52; "Reflections on the History of Social Policy in America," ibid., vii (1979), 293–306; "Doing Good and Getting Worse: The Dilemma of Social Policy," *Michigan Law Review*, lxxvii (1979), 761–783; "Rediscovering Asylums: The Unhistorical History of the Mental Hospital" in *The Therapeutic Revolution: Essays in the Social History of American Medicine*, edited by Morris Vogel and Charles E. Rosenberg (Philadelphia: University of Pennsylvania Press, 1979), 135–157; "Marxian Analysis and Mental Illness," *History of Psychiatry*, i (1990), 223–232.

27. See Grob, "The Origins of American Psychiatric Epidemiology," *American Journal of Public Health*, lxxv (1985), 229–236, and "Origins of DSM-I: A Study in Appearance and Reality," *American Journal of Psychiatry*, cxxxxviii (1991): 421–431.

28. To maintain that psychiatric and medical nosologies are not fundamentally dissimilar is not to reject the view that the concept of disease is in part socially constructed and hence dependent in part on nonscientific and external variables. With the possible exception of a number of infectious diseases, most pathological states are still described in terms of symptoms rather than etiology. Admittedly, there is a difference between defining disease in terms of behavioral signs, on the one hand, and physiological symptoms, on the other. But that difference may not be of fundamental significance, particularly if little or no distinction is made between behavior and physiological processes. In fact, knowledge about "mental illnesses" is still extremely limited. Moreover, in many cases it is still not feasible to establish the validity of psychiatric disease categories. Schizophrenia, for example, may be, in fact, what "fever" was before 1870—a general inclusive category that comprehends a multiplicity of diseases. Assertions about the existence or nonexistence of mental illnesses represent, in part, acts of faith, which in turn reflect commitments to particular explanations and interventions.

29. E. A. Ross, *Social Control: A Survey of the Foundations of Social Order* (New York: Macmillan, 1901), and Helen Everett, "Control, Social" in *Encyclopedia of the Social Sciences*, vol. 4 (New York: Macmillan, 1930–1935), 344—both cited in James Leiby, "Social Control and Historical Explanation: Historians View the Piven and Cloward Thesis" in *Social Welfare or Social Control? Some Historical Reflections on Regulating the Poor*, edited by Walter I. Trattner (Knoxville: University of Tennessee Press, 1983), 97–98.

30. Rothman, *Discovery of the Asylum*, "Introduction to the 1990 Edition," xxxviff.; Scull, "Psychiatry and Social Control in the Nineteenth and Twentieth Centuries," *History of Psychiatry*, ii (1991), 159–169.

31. For sophisticated and nonpejorative uses of the concept of social control see Morris Janowitz, "Sociological Theory and Social Control," *American Journal of Sociology*, lxxxi (1975), 82–108; idem, *On Social Organization and Social Control* (Chicago: University of Chicago Press, 1991); and Jack P. Gibbs, *Control: Sociology's Central Notion* (Urbana: University of Illinois Press, 1989).

32. Grob, *The State and the Mentally Ill*, 252–255; Nancy Tomes, *A Generous Confidence: Thomas Story Kirkbride and the Art of Asylum-keeping, 1840–1883* (New York: Cambridge University Press, 1984); Ellen Dwyer, *Homes for the Mad: Life Inside Two Nineteenth-Century Asylums* (New Brunswick: Rutgers University Press, 1987); Constance M. McGovern, "The Myths of Social Control and Custodial Oppression: Patterns of Psychiatric Medicine in Late Nineteenth-Century Institutions," *Journal of Social History*, xx (1986), 3–23; Morton Kramer et al., *A Historical Study of First Admissions to a State Mental Hospital: Experience of the Warren State Hospital during the Period 1916–50* (Washington, D. C.: Government Printing Office, 1955), 12.

33. Dwyer, *Homes for the Mad*, 86ff.; Richard W. Fox, *So Far Disordered in Mind: Insanity in California, 1870—1930* (Berkeley: University of California Press, 1978), 84ff.; Tomes, *A Generous Confidence*, 109.

34. J. H. Plumb, *The Death of the Past* (Boston: Houghton Mifflin, 1970), 13, 17.